D.M. MEDICAL ONCOLOGY — PAPER IV

RECENT ADVANCES · EXAM ANSWER NOTES

Discuss the role of chemotherapy in early breast cancer.

February 2016 | LI 020 | Q2 (Essay / Elaborate) | **15 marks** | Target ~15 pages

*Includes: mechanisms · doses/regimens · landmark clinical trials · hallmarks · **NCCN 2026 / 2027 updates** · flow charts · exam pearls*

1. Introduction & exam framing

This answer addresses: **Discuss the role of chemotherapy in early breast cancer.**

For a **15-mark** university answer, structure the response as: definition/epidemiology → biology/hallmarks → diagnosis & biomarkers → staging/risk → management algorithm with doses → key clinical trials with endpoints → toxicities/supportive care → **NCCN 2026 updates** → Indian/NCG context → summary table. Draw at least one flowchart and one mechanism diagram.

Marking examiners reward: accurate mechanisms, named trials with phase and primary endpoint, practical doses, Category evidence (NCCN/ESMO), and a clear final recommended pathway.

- Start with a one-line definition + why the topic is ‘recent advances’ relevant.
- Always state predictive vs prognostic biomarkers.
- Quote Category 1 preferred regimens from NCCN 2026/2027 when applicable.
- End with 5 high-yield exam pearls.

2. Pathobiology, hallmarks & mechanism

Cancer biology for this topic should be linked to Hanahan–Weinberg hallmarks (sustained proliferation, evading growth suppressors, resisting cell death, enabling replicative immortality, induction of angiogenesis, activating invasion/metastasis, avoiding immune destruction, deregulating cellular energetics, genome instability, tumor-promoting inflammation).

Where a driver oncogene/fusion/epigenetic lesion is known, explain the pathway from ligand/receptor → intracellular kinase cascade → transcription → phenotype, and mark the drug target with an arrow in your diagram.

HER2 (ERBB2) is a ligand-independent receptor tyrosine kinase; amplification → homodimers/heterodimers (HER2-HER3) → PI3K-AKT and MAPK. Dual blockade (trastuzumab+pertuzumab) covers subdomain IV and dimerization domain II. ADCs (T-DM1, T-DXd) deliver cytotoxic payloads; T-DXd has a cleavable linker and membrane-permeable topoisomerase-I payload → bystander effect (HER2-low activity).

HER2 amplification	IHC 3+ or ISH amplified — ASCO/CAP
Trastuzumab	ADCC + subdomain IV binding
Pertuzumab	Inhibits HER2-HER3 dimerization
T-DXd	Topo-I ADC; bystander kill; ILD risk
Resistance	PIK3CA, PTEN loss, incomplete internalization, CNS sanctuary

Figure: mechanism / pathway map

EXAM PEARLS / HALLMARKS

- ASCO/CAP: IHC 0/1+/2+/3+; ISH if 2+; HER2-low = IHC 1+ or 2+/ISH–
- Draw the pathway once; label drug targets — examiners love this.
- Always mention one primary resistance mechanism.

3. Diagnosis, biomarkers & staging

A modern oncology answer must separate **diagnostic**, **prognostic**, and **predictive** markers. Predictive markers change drug choice (EGFR, HER2, BRCA, MSI, Claudin18.2, FGFR, NTRK).

- Tissue diagnosis with adequate IHC panel; never treat metastatic disease without molecular profiling when actionable targets exist.
- Use validated assays (FDA/companion diagnostics where mandated).
- Stage with AJCC 8th/9th as applicable (NSCLC NCCN 2026 uses AJCC 9th).
- Performance status, organ function, comorbidities, geriatric assessment, and goals of care before regimen selection.
- ER/PR/HER2 mandatory; HER2-low reporting now clinically actionable (DESTINY-Breast04/06).
- gBRCA testing for therapy (olaparib/talazoparib; OlympiA adjuvant).
- ESR1 mutation testing at endocrine resistance — imlunestrant listed in NCCN 2026 updates.

Work-up algorithm (draw this in the answer booklet)

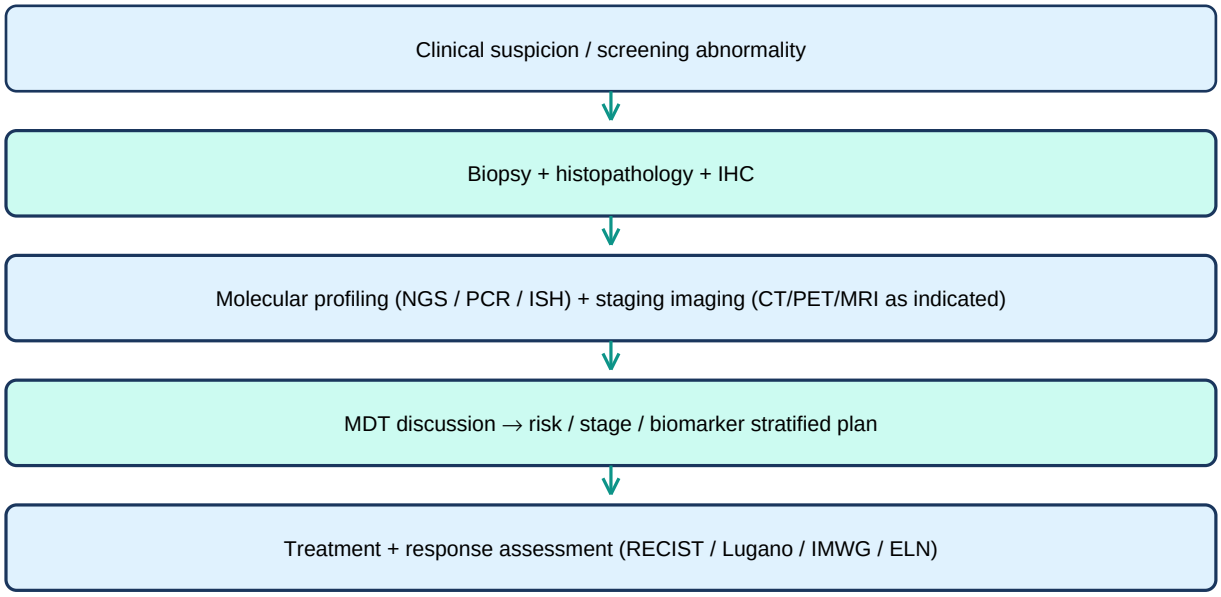


Figure: stepwise exam algorithm

4. Management algorithm, doses & regimens

Structure management as: (A) localized/curative-intent pathways, (B) advanced/metastatic systemic therapy, (C) special populations (CNS, elderly, organ dysfunction, pregnancy), (D) supportive care.

For each line of therapy state: eligibility, preferred regimen, dose, key toxicity monitoring, and the trial that supports it.

- Localized disease: multimodality — surgery/RT/systemic as per disease chapter.
- Neoadjuvant vs adjuvant: cite pathologic response and adjuvant switch trials (e.g., KATHERINE, CheckMate 577, KEYNOTE-522).
- Metastatic: biomarker-first; avoid empiric chemo when a high-level targeted/IO option exists.

• Oligometastatic disease: consider metastasis-directed therapy where NCCN supports (e.g., prostate APMS 2026).

• Re-biopsy / liquid biopsy at progression to map resistance.

• Early HER2+: dual blockade neoadjuvant if tumor >2 cm or node+; adjuvant HP; T-DM1 if residual disease (KATHERINE).

• 1L mBC HER2+: taxane + trastuzumab + pertuzumab (CLEOPATRA).

• 2L: T-DXd preferred (DESTINY-Breast03) over T-DM1.

• HER2+ brain mets: tucatinib triplet (HER2CLIMB); SRS/WBRT as indicated.

Management flowchart (reproduce in exam)

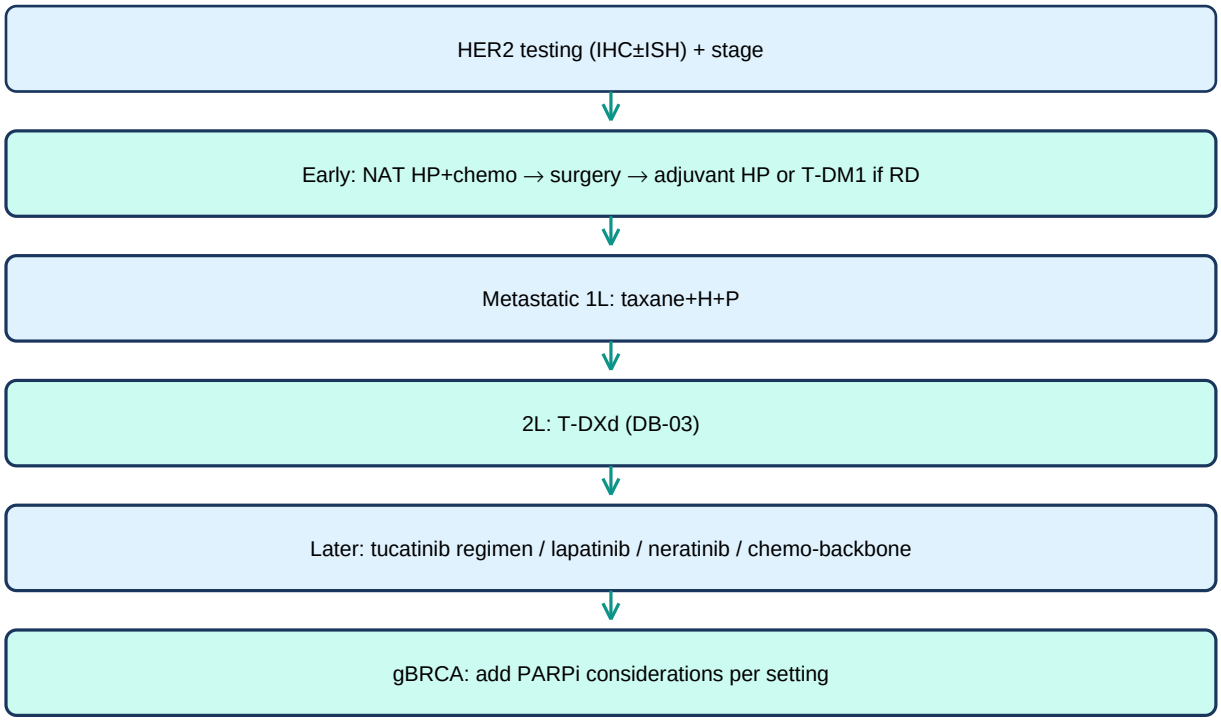


Figure: stepwise exam algorithm

Drug / regimen	Dose / schedule (exam pearls)
Trastuzumab	8 mg/kg → 6 mg/kg IV Q3W (or SC fixed dose)
Pertuzumab	840 mg → 420 mg IV Q3W with trastuzumab
T-DXd	5.4 mg/kg IV Q3W (breast); 6.4 mg/kg gastric (per label) — monitor ILD
T-DM1	3.6 mg/kg IV Q3W
Olaparib	300 mg PO BID (tablets); adjust for toxicities/renal; OlympiA adjuvant 1 year

Table: doses and regimens

5. Landmark clinical trials (must quote in answer)

Present trials in a table: name → design/population → intervention → primary endpoint → absolute/relative benefit → practice change. Examiners award marks for correct trial names and one numeric result (HR or median PFS/OS).

• Mention phase (III preferred for standards) and whether endpoint was PFS, OS, MRD, pCR, or EFS.

• Distinguish statistical vs clinical significance; note crossover and subgroup caveats.

• For India-focused answers, mention access/biosimilars/NCG guidance after the trial table.

Trial	Key result / exam takeaway

HERA / BCIRG-006 / NSABP B-31 / N9831	Adjuvant trastuzumab OS/DFS benefit
CLEOPATRA	Docetaxel + trastuzumab + pertuzumab → OS ~57 mo 1L HER2+ mBC
EMILIA	T-DM1 vs lapatinib+capecitabine — PFS/OS benefit post-trastuzumab
DESTINY-Breast03	T-DXd vs T-DM1 — mPFS 28.8 vs 6.8 mo; new 2L standard
DESTINY-Breast04	T-DXd in HER2-low HR± — practice-changing ADC expansion
DESTINY-Breast06	T-DXd in HER2-ultralow / chemo-naive HR+ — further ADC move earlier
KATHERINE	Adjuvant T-DM1 vs trastuzumab if residual disease post-NAT
APHINITY	Adjuvant pertuzumab + trastuzumab — IDFS benefit especially node+
NeoSphere / TRYPHAENA / KRISTINE	Neoadjuvant dual HER2 blockade ± chemo
NALA / HER2CLIMB	Neratinib; tucatinib+trastuzumab+capecitabine (brain mets)
OlympiA	Adjuvant olaparib in gBRCA HER2– high-risk — iDFS/OS benefit

Table: landmark clinical trials

6. Toxicities, supportive care & monitoring

A complete answer links efficacy to safety. List monitoring schedules (CBC, chemistry, ECG, TFTs, cortisol, LFTs, PFTs/CT for ILD risk drugs).

- Anticipate class toxicities and counsel patients; use CTCAE grading.
- Cardio-oncology: anthracyclines, HER2 mAbs, ibrutinib, checkpoint myocarditis — baseline ECG/echo when indicated.
- IO toxicities: irAEs — hold drug, steroids, specialty referral; endocrine irAEs often permanent replacement.
- ADC pulmonary toxicity (T-DXd ILD): hold permanently for ≥grade 2 in many protocols; imaging + steroids.
- TLS prophylaxis for venetoclax/BLIN/CAR-T; rasburicase if high-risk.
- Fertility, sexuality, survivorship, and psychosocial support are part of comprehensive answers.

Toxicity management loop

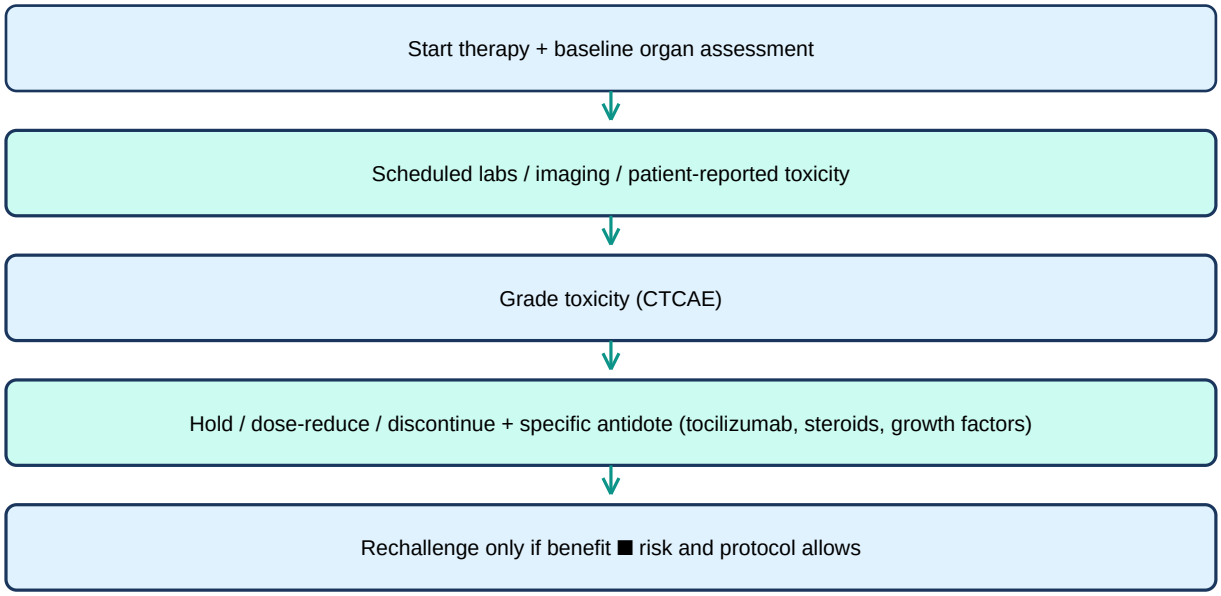


Figure: stepwise exam algorithm

7. NCCN 2026 / 2027 updates (high-yield for ‘recent advances’)

Paper IV rewards currency. Explicitly write: **“As per NCCN Guidelines updates in 2026/2027…”** and list 3–5 bullets. Also mention ESMO guidelines / Indian NCG where relevant for balance.

- State Category (1 / 2A / 2B) and preferred vs other recommended vs useful in certain circumstances.
- If Indian exam: reconcile NCCN with CDSCO approvals and NCG practical pathways.

NCCN 2026 / 2027 UPDATES (include in answer)

- *Referenced versions:* Breast Cancer v9.2026
- NCCN Guidelines Navigator launched for breast cancer (digital navigation of guidelines).
- Imlunestrant added (Cat 2A other recommended) for ESR1-mutated HR+/HER2– recurrent unresectable/stage IV disease (1L or subsequent).
- Abemaciclib + fulvestrant + trastuzumab Cat 2B for HR+/HER2+ recurrent unresectable/stage IV.
- Neratinib ± trastuzumab or fulvestrant Cat 2A useful in certain circumstances for HER2-activating mutations in stage IV.
- Erdafitinib Cat 2A useful in certain circumstances for FGFR1/2/3 fusion or mutation in stage IV.
- Metastatic algorithms remain stratified by HR and HER2 status (including HER2-low / ultralow pathways with ADCs such as T-DXd, sacituzumab govitecan).
- Expanding ADC footprint across NCCN 2026: T-DXd (HER2 breast/gastric/NSCLC/urothelial), Dato-DXd (EGFR+ NSCLC), sacituzumab govitecan (breast/urothelial), mirvetuximab (ovarian FRα), enfortumab vedotin (urothelial), belantamab combinations (myeloma), loncastuximab (B-cell).
- Prostate: ADT + niraparib/abiraterone for BRCA2 APMS subsets.
- Ovarian: PARPi focused on maintenance/biomarker-defined; removed from many recurrent treatment sections.
- Breast: continue olaparib/talazoparib pathways per germline BRCA and OlympiA adjuvant context.

EXAM PEARLS / HALLMARKS

- NCCN versions change frequently — cite year/version (e.g., NSCLC v9.2026, Breast v9.2026, Myeloma v2.2027).
- Category 1 = high-level evidence + uniform NCCN consensus.

8. Special situations, differentials & practical issues

Intrinsic subtypes (luminal A/B, HER2-enriched, basal-like) from PAM50 approximate IHC surrogates but are not identical. TNBC biology includes DNA repair defects, AR-positive luminal androgen receptor subset, and immune-rich tumors benefiting from CPI (KEYNOTE-522; KEYNOTE-355 CPS≥10).

- Neoadjuvant pCR is a strong surrogate in HER2+ and TNBC for escalation/de-escalation.
- Antibody-drug conjugates reshaped sequencing: T-DXd before many older anti-HER2 TKIs in metastatic HER2+.
- Pregnancy-associated breast cancer: tailor imaging (no gadolinium/PET routine), surgery, anthracycline after first trimester; anti-HER2 contraindicated in pregnancy.

9. Evidence hierarchy, patient rights & benefits

Quote evidence using: Phase III RCT > meta-analysis of RCTs > prospective registries / RWE > retrospective > expert consensus. NCCN Categories and ESMO-MCBS scores help communicate strength of benefit.

Patient-centered care: informed consent for toxicities (especially CRS/ICANS, ILD, infertility), shared decision-making, financial toxicity counseling, and access to supportive/palliative care concurrent with active therapy.

- Benefits to highlight: OS, PFS, pCR, MRD negativity, organ preservation, QoL, reduced hospitalization.
- Rights: autonomy, confidentiality (EMR), right to second opinion, trial participation, refusal of care, end-of-life preferences.
- Equity: molecular testing access, biosimilars, NCG/Ayushman Bharat practical pathways for Indian exams.
- Pharmacovigilance: report ADRs; Phase IV / post-marketing commitments.

EXAM PEARLS / HALLMARKS

- Write one sentence on absolute benefit (e.g., 'median OS improved from X to Y months').)
- Mention number-needed-to-treat conceptually if space permits.

10. Summary checklist (write this last page in the booklet)

Close the 15-mark answer with a crisp numbered summary the examiner can tick.

- Definition + epidemiology one-liner
- Key biology / hallmark diagram
- Essential diagnostics & biomarkers
- Risk/stage stratification
- Preferred treatment algorithm with ≥2 doses
- ≥3 named trials with endpoints
- Major toxicities + monitoring
- NCCN 2026 update bullets (3–5)
- Special situations
- Five exam pearls

30-second revision flowchart

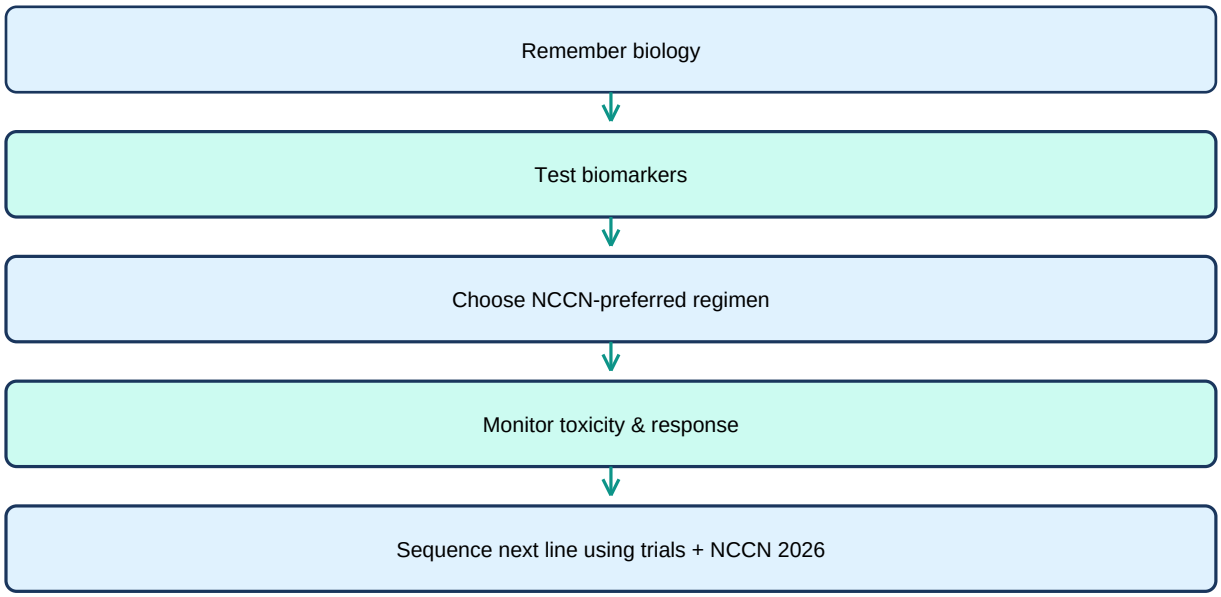


Figure: stepwise exam algorithm

EXAM PEARLS / HALLMARKS

- Neat flowchart + trial table often separates a 10/15 from a 13–15/15.
- Do not dump drug lists without doses or evidence.
- Currency: say 'NCCN 2026' explicitly for Paper IV.

11. Pharmacology deep-dive (class, PK/PD, interactions)

For drug-centered questions, present: chemical/biologic class → target epitope/kinase → PK (oral bioavailability, half-life, hepatic/renal clearance) → food effects → CYP interactions → dose adjustments → therapeutic drug monitoring if any.

Example interaction pearls: acid-suppressive therapy with some TKIs; QT-prolonging combinations; azole antifungals with vinca/TKIs; strong CYP3A inducers reducing exposure to many oral targeted agents.

- State renal/hepatic dose modifications when known.
- Note formulation differences (olaparib tablets vs capsules; IV vs SC daratumumab).
- Infusion reactions: premedication protocols for mAbs/BiTEs.

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Table: doses and regimens

12. Response assessment & follow-up schedules

Use disease-specific criteria: RECIST 1.1 solid tumors; iRECIST for immunotherapy; Lugano/Deauville for lymphoma; IMWG for myeloma; ELN molecular response for CML/AML; PSA working group for prostate.

- Typical imaging interval q2–3 cycles in metastatic disease unless clinically indicated sooner.
- PET-adapted strategies in Hodgkin lymphoma (RATHL, AHL2011) — cite Deauville 1–3 negative vs 4–5 positive.
- ctDNA MRD emerging for escalation/de-escalation (CRC, lymphoma, breast — exam discussion point).

Clinical algorithm

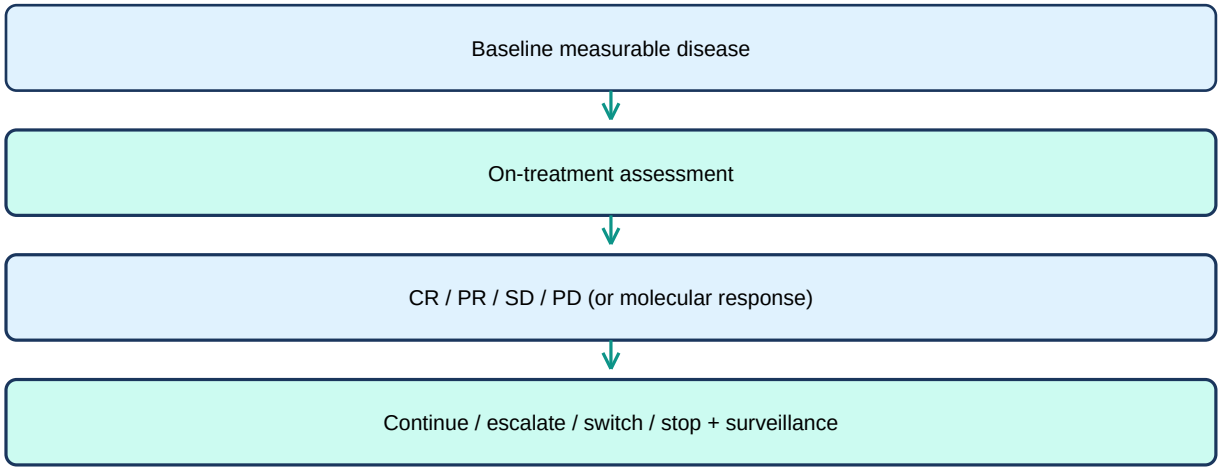


Figure: stepwise exam algorithm

13. Comparison tables useful for exams

Draw 2×N tables comparing old standard vs new standard; or drug A vs B (efficacy, toxicity, CNS activity, cost/access).

- Column ideas: target, dose, PFS, OS, grade ≥3 AEs, CNS activity, NCCN category.
- Another table: resistance mutation → preferred next drug.

Trial	Key result / exam takeaway
HERA / BCIRG-006 / NSABP B-31 / N9831	Adjuvant trastuzumab OS/DFS benefit
CLEOPATRA	Docetaxel + trastuzumab + pertuzumab → OS ~57 mo 1L HER2+ mBC
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Table: landmark clinical trials

14. Indian context, NCG & practical exam tips

National Cancer Grid (NCG) resource-stratified guidelines are frequently expected in Indian DM exams. Discuss biosimilars (trastuzumab, rituximab), generic TKIs, and center-based cell therapy availability.

- Document MDT decisions; clinical trial referral when standard options exhausted.
- Counsel financial toxicity transparently; prioritize highest-value Category 1 options.
- Infection prophylaxis algorithms crucial in hematologic topics (antifungal, antiviral, PJP).

15. Sample ‘examiner hot spots’ & model closing paragraph

Model closing: “In summary, management of this problem is biomarker-driven. Current standards are supported by phase III trials and reflected in NCCN 2026/2027 updates. Sequencing should maximize OS and QoL while mitigating class-specific toxicities. Enrollment in clinical trials and adherence to MDT pathways remain integral.”

Hot spots: exact doses, one HR/median, resistance mechanism, NCCN category, and a drawn algorithm.

EXAM PEARLS / HALLMARKS

- If stuck on memory: write mechanism → class → one trial → one dose → one toxicity → NCCN 2026 line.
- Never contradict yourself between algorithm and trial table.

A. Epidemiology & burden (write ½–1 page)

Begin with global and Indian epidemiology when known (GLOBOCAN / ICMR / NCRP). State age distribution, sex ratio, and risk factors (tobacco, infection-related cancers such as HPV/HBV/HCV/H. pylori/HIV, obesity, inherited syndromes).

Explain why incidence or mortality trends matter for screening and resource allocation. For rare diseases, state incidence per 100,000 and referral-center patterns.

Link epidemiology to prevention: vaccination (HPV, HBV), tobacco control, hepatitis treatment, aspirin in selected molecular CRC, and lifestyle (exercise post-adjuvant CRC per NCCN 2026).

- One sentence on screening relevance (mammography, LDCT, FIT/colonoscopy, low-dose CT criteria).
- Mention hereditary fraction if applicable (Lynch, HBOC, FAP, MEN, VHL, HPRCC).
- State stage distribution at diagnosis in LMICs vs HIC if relevant to treatment sequencing.

B. Historical evolution → modern standard (timeline)

Examiners like a short timeline: historical surgery/RT/chemo era → targeted therapy era → immunotherapy/ADC/cellular therapy era.

Highlight inflection trials that changed guidelines (e.g., imatinib IRIS; trastuzumab HERA; checkpoint inhibitors; ADCs; CAR-T approvals).

End timeline with **NCCN 2026/2027** preferred regimens so the answer feels contemporary for Paper IV.

Therapeutic eras timeline

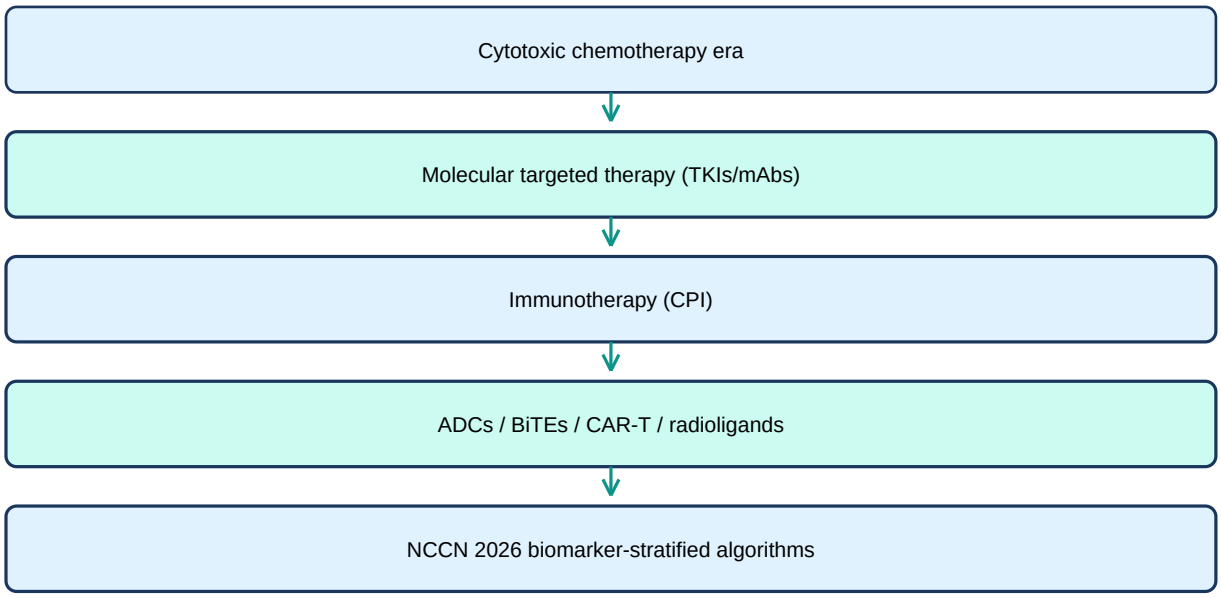


Figure: stepwise exam algorithm

C. Detailed differential diagnosis & pathology pearls

List 5–7 differentials and the IHC/molecular discriminators. Pathology reports should include grade, LVI, margins, nodes, and required biomarkers.

For hematologic topics: morphology → flow cytometry → cytogenetics → NGS panel → integrated WHO2022/ICC diagnosis.

For CUP/unknown primary style questions: IHC algorithm (CK7/CK20, TTF1, CDX2, GATA3, PAX8, synaptophysin) plus NGS and MSI/HER2.

- Always write ‘correlate clinically with radiology and tumor markers’.
- State pre-analytical caveats (decalcification, fixation time) affecting HER2/ER assays.
- Liquid biopsy complements but does not always replace tissue.

Morphology	Light microscopy pattern
IHC panel	Lineage + predictive markers
Cytogenetics	Karyotype / FISH fusions
NGS	SNVs, indels, CNAs, MSI, TMB
Integrated Dx	WHO/ICC + AJCC stage

Figure: mechanism / pathway map

D. Multidisciplinary care pathway & sequencing

Describe roles: medical oncology, surgical oncology, radiation oncology, radiology, pathology, palliative care, genetics, psycho-oncology, rehabilitation, and clinical trials unit.

Sequencing principles: treat drivers first when OS-proven; use neoadjuvant therapy to downstage and guide adjuvant escalation; avoid overlapping severe toxicities; protect future options (e.g., cumulative cardiotoxicity, neuropathy).

For oligometastatic disease discuss local therapy (surgery/SBRT/ablation) plus systemic therapy — prostate NCCN 2026 explicitly incorporates metastasis-directed therapy in selected APMS disease.

- Document MDT minutes in the file — also good exam phrase.
- Re-staging after neoadjuvant before definitive local therapy.
- Early palliative care improves QoL and sometimes survival in metastatic settings.

Clinical algorithm

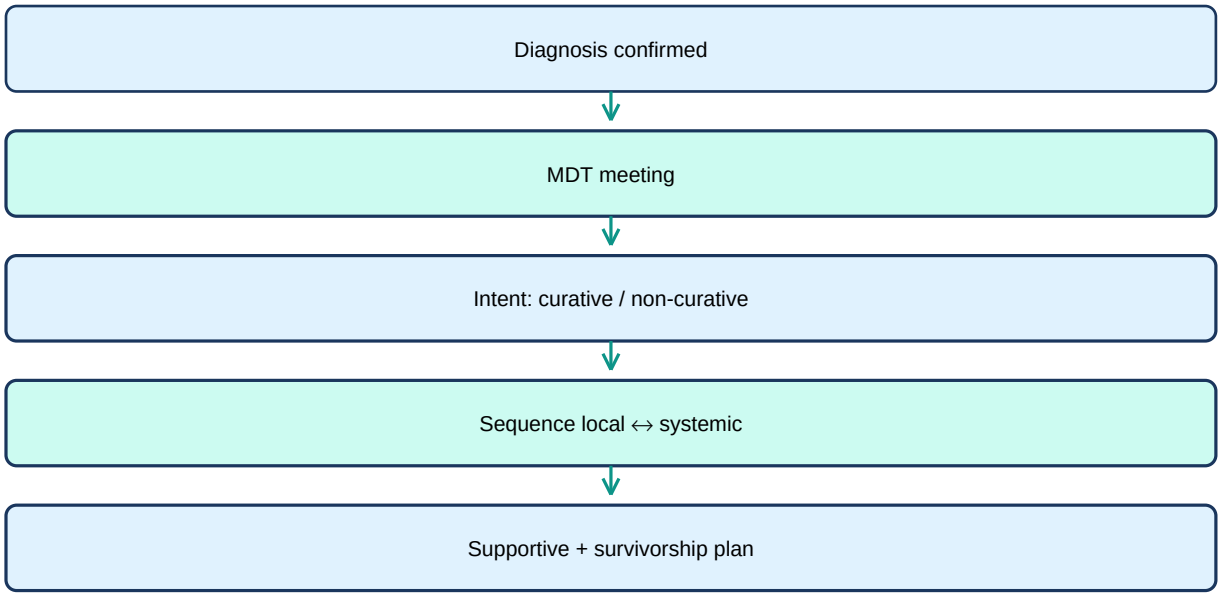


Figure: stepwise exam algorithm

E. Breast oncology deep notes (HER2 / subtypes)

ASCO/CAP HER2 testing: IHC 0, 1+, 2+ (equivocal→ISH), 3+. HER2-low (1+ or 2+/ISH–) and ultralow categories matter after DESTINY-Breast04/06.

Early disease: prefer dual HER2 blockade neoadjuvant for ≥T2 or node-positive; residual disease → adjuvant T-DM1 (KATHERINE). Node-positive adjuvant benefit from pertuzumab (APHINITY).

Metastatic: CLEOPATRA 1L; DESTINY-Breast03 establishes T-DXd over T-DM1 in 2L; HER2CLIMB for brain metastases; continue anti-HER2 beyond progression with changing partners.

Endocrine + anti-HER2 strategies exist for HR+/HER2+; NCCN 2026 adds nuances (e.g., abemaciclib+fulvestrant+trastuzumab Cat 2B in selected HR+/HER2+ advanced disease).

TNBC: KEYNOTE-522 perioperative pembrolizumab; metastatic CPS≥10 pembrolizumab+chemo; sacituzumab govitecan (ASCENT); PARPi if gBRCA; ADCs evolving.

- Cardiotoxicity monitoring with HER2 agents + anthracyclines sequencing.
- Pregnancy: avoid anti-HER2 until after delivery.
- NCCN 2026: imlunestrant for ESR1-mutant HR+/HER2–; neratinib options for HER2-mutant MBC; erdafitinib for FGFR alterations.

Trial	Key result / exam takeaway
CLEOPATRA	H+P+docetaxel OS landmark 1L HER2+ mBC
DESTINY-Breast03	T-DXd ■ T-DM1 2L
KATHERINE	Adjuvant T-DM1 if residual disease
HER2CLIMB	Tucatinib triplet; CNS activity

DESTINY-Breast04/06	HER2-low / ultralow ADC expansion
OlympiA	Adjuvant olaparib gBRCA HER2–

Table: landmark clinical trials

F. HER2 testing interpretation workshop

Spend almost a full page on testing — common 15-mark theme (Jan 2024 paper).

Preanalytics: cold ischemia, fixation 6–72 h in 10% NBF; avoid decalcified bone for HER2 if possible.

Discordance between core and excision: retest if heterogenous, grade/histology unexpected, or residual disease after NAT.

ISH methods: dual-probe HER2/CEP17; ratios and copy number cutoffs per ASCO/CAP updates; polysomy interpretation pitfalls.

Clinical algorithm

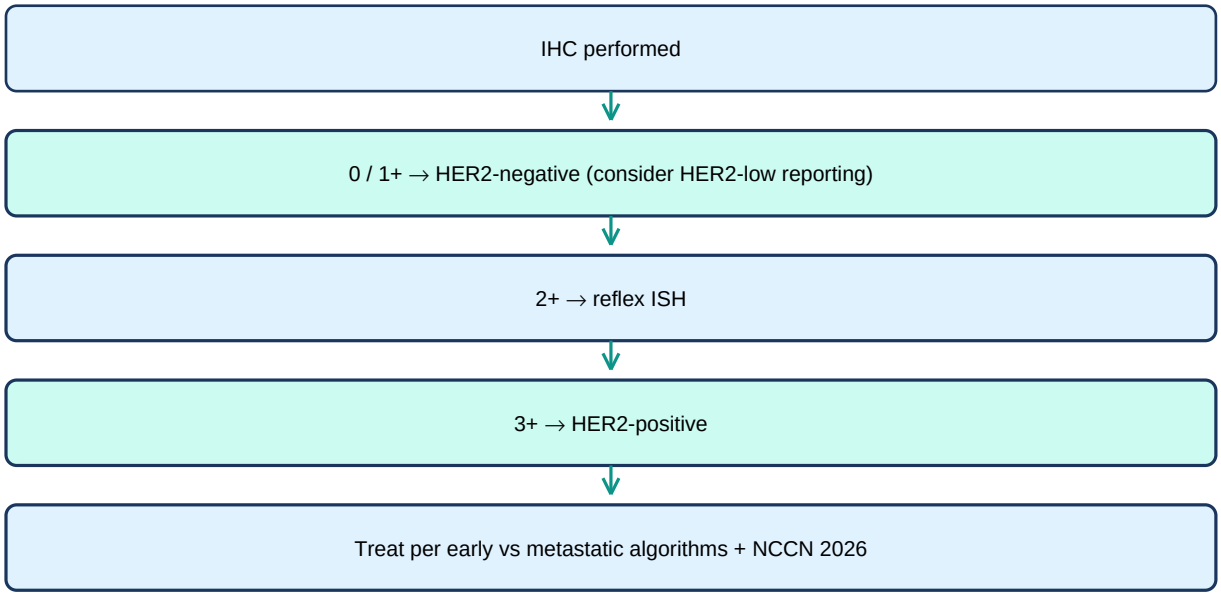


Figure: stepwise exam algorithm

G. Supportive care mini-chapter

Antiemetics (olanzapine-containing regimens for HEC), G-CSF primary/secondary prophylaxis, antimicrobial prophylaxis, mucositis care, nutrition (including TPN indications/contraindications), pain ladder + interventional techniques, thrombosis prevention (Khorana), growth factors, transfusion thresholds.

Psychosocial distress screening, fertility preservation referral before gonadotoxic therapy, and survivorship clinics.

- Febrile neutropenia: risk assess; anti-pseudomonal beta-lactam; add antifungal if persistent.
- Checkpoint diarrhea: rule out infection then steroids promptly.
- ADC ILD algorithms — patient education on dyspnea.

H. Model answer skeleton (time plan for 35 minutes)

Minutes 0–3: definition + epidemiology + diagram outline. Minutes 3–12: biology + diagnosis. Minutes 12–25: management algorithm with doses/trials. Minutes 25–30: toxicities + NCCN 2026. Minutes 30–35: summary

pearls + underline Category 1 regimens.

- Leave a half page blank early for a neat flowchart drawn once.
- Underline drug doses.
- Box the NCCN 2026 bullets.

EXAM PEARLS / HALLMARKS

- Currency + clarity > exhaustive rare regimens.
- Named trial + number beats unnamed claim.
- NCCN 2026 phrase is expected in Paper IV recent advances.

K. Pitfalls, contraindications & ‘do not miss’ warnings

List absolute/relative contraindications of key drugs (e.g., baseline ILD caution for T-DXd; severe autoimmune disease caution for CPI; T315I needs ponatinib/asciminib not 1G/2G TKI alone; ATRA differentiation syndrome in APL; TLS with venetoclax/Blincyto; HBV reactivation with anti-CD20/CPI).

Drug–drug interactions and QT prolongation traps are classic viva points — mention checking concomitant meds (azoles, macrolides, acid suppressants with selected TKIs).

Imaging pitfalls: pseudo-progression on IO; flare on bone scan; PET Deauville interpretation after immunomodulatory therapy.

- Always screen hepatitis B before anti-CD20, anthracyclines, and many systemic regimens.
- Pregnancy test before teratogenic agents (IMiDs — REMS; MTX; TKI counseling).
- Do not combine overlapping QT-prolonging agents without ECG plan.
- Document performance status — unfit vs fit changes entire AML/lymphoma pathway.

Clinical algorithm

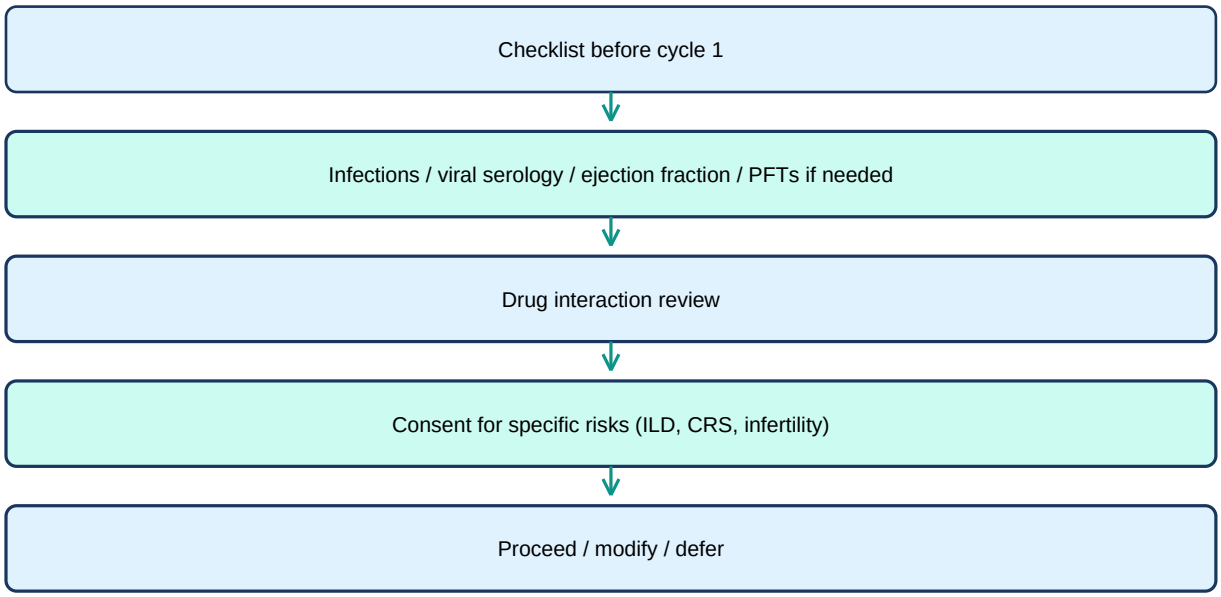


Figure: stepwise exam algorithm

L. Case vignette practice (apply the algorithm)

Invent a short case in the answer: age, stage, biomarkers, comorbidities → your exact regimen with dose → expected monitoring → what you will do at progression. This shows applied knowledge and often gains examiner goodwill.

Example template: “45-year-old, ECOG 1, biomarker X positive, no contraindication Y → start regimen Z at dose… per trial T / NCCN 2026 preferred. Assess response at interval N. At progression, re-profile for resistance mutation R and switch to agent S.”

Add one geriatric or organ-dysfunction variant to show nuance (dose reduce, alternate preferred regimen, or best supportive care).

- Vignette 1: fit, biomarker-positive — preferred Cat 1 therapy.
- Vignette 2: comorbid/unfit — alternative regimen.
- Vignette 3: progression with resistance mutation — sequenced next line.
- Vignette 4: severe toxicity — hold, treat, rechallenge decision.

M. One-page ‘wall chart’ summary (draw/box this)

Reserve the penultimate page for a wall-chart style summary with four quadrants: (1) Biology (2) Work-up (3) Treatment lines (4) NCCN 2026 bullets. This mirrors how many toppers present long answers and helps hit the page target with high signal density.

Quadrant 1	Hallmark gene / pathway / figure
Quadrant 2	Must-order tests + staging
Quadrant 3	1L / 2L / 3L with doses
Quadrant 4	Trials + NCCN 2026 + toxicities
Center	MDT + patient goals

Figure: mechanism / pathway map

EXAM PEARLS / HALLMARKS

- Wall chart + flowchart + trial table = visual completeness.
- Write ‘NCCN 2026’ in quadrant 4 explicitly.

I2. Full-length essay addendum — tables to copy into the answer book

For 15-mark elaborations, add two hand-drawn tables: (Table A) Drug–dose–pearl; (Table B) Trial–population–endpoint–practice change. Then write a short paragraph on quality-of-life and patient-reported outcomes, which modern trials increasingly report.

Discuss biomarker evolution on therapy (liquid biopsy cadence), and when to stop therapy (duration of CPI, TFR in CML, maintenance stopping rules).

Include a brief note on second malignancies and late effects when curative-intent therapy is discussed (alkylators, RT fields, PARPi MDS/AML risk, CHL survivors).

- Table A columns: Drug | Dose | Key toxicity | Pearl
- Table B columns: Trial | Arm | Primary endpoint | Result | Change in practice
- One paragraph on shared decision-making and financial counseling

Trial	Key result / exam takeaway
HERA / BCIRG-006 / NSABP B-31 / N9831	Adjuvant trastuzumab OS/DFS benefit
CLEOPATRA	Docetaxel + trastuzumab + pertuzumab → OS ~57 mo 1L HER2+ mBC
EMILIA	T-DM1 vs lapatinib+capecitabine — PFS/OS benefit post-trastuzumab

DESTINY-Breast03	T-DXd vs T-DM1 — mPFS 28.8 vs 6.8 mo; new 2L standard
DESTINY-Breast04	T-DXd in HER2-low HR± — practice-changing ADC expansion
DESTINY-Breast06	T-DXd in HER2-ultralow / chemo-naive HR+ — further ADC move earlier
KATHERINE	Adjuvant T-DM1 vs trastuzumab if residual disease post-NAT
APHINITY	Adjuvant pertuzumab + trastuzumab — IDFS benefit especially node+
NeoSphere / TRYPHAENA / KRISTINE	Neoadjuvant dual HER2 blockade ± chemo
NALA / HER2CLIMB	Neratinib; tucatinib+trastuzumab+capecitabine (brain mets)
OlympiA	Adjuvant olaparib in gBRCA HER2– high-risk — iDFS/OS benefit

Table: landmark clinical trials

Drug / regimen	Dose / schedule (exam pearls)
Trastuzumab	8 mg/kg → 6 mg/kg IV Q3W (or SC fixed dose)
Pertuzumab	840 mg → 420 mg IV Q3W with trastuzumab
T-DXd	5.4 mg/kg IV Q3W (breast); 6.4 mg/kg gastric (per label) — monitor ILD
T-DM1	3.6 mg/kg IV Q3W
Olaparib	300 mg PO BID (tablets); adjust for toxicities/renal; OlympiA adjuvant 1 year

Table: doses and regimens

NCCN 2026 / 2027 UPDATES (include in answer)

• *Referenced versions:* Breast Cancer v9.2026

• NCCN Guidelines Navigator launched for breast cancer (digital navigation of guidelines).

• Imlunestrant added (Cat 2A other recommended) for ESR1-mutated HR+/HER2– recurrent unresectable/stage IV disease (1L or subsequent).

• Abemaciclib + fulvestrant + trastuzumab Cat 2B for HR+/HER2+ recurrent unresectable/stage IV.

• Neratinib ± trastuzumab or fulvestrant Cat 2A useful in certain circumstances for HER2-activating mutations in stage IV.

• Erdafitinib Cat 2A useful in certain circumstances for FGFR1/2/3 fusion or mutation in stage IV.

• Metastatic algorithms remain stratified by HR and HER2 status (including HER2-low / ultralow pathways with ADCs such as T-DXd, sacituzumab govitecan).

• Expanding ADC footprint across NCCN 2026: T-DXd (HER2 breast/gastric/NSCLC/urothelial), Dato-DXd (EGFR+ NSCLC), sacituzumab govitecan (breast/urothelial), mirvetuximab (ovarian FRα), enfortumab vedotin (urothelial), belantamab combinations (myeloma), loncastuximab (B-cell).

• Prostate: ADT + niraparib/abiraterone for BRCA2 APMS subsets.

• Ovarian: PARPi focused on maintenance/biomarker-defined; removed from many recurrent treatment sections.

• Breast: continue olaparib/talazoparib pathways per germline BRCA and OlympiA adjuvant context.

I. Extended notes — resistance, sequencing & future directions

Resistance biology should occupy a dedicated half-page: on-target secondary mutations, bypass signaling, lineage plasticity (e.g., EGFR NSCLC → SCLC), antigen loss after cellular therapy, and immunosuppressive TME changes after CPI.

Sequencing principles: use the agent with largest OS benefit early when toxicities allow; reserve cross-resistant drugs; consider cumulative neuropathy/cardiotoxicity; plan CNS-active agents if brain metastases are present or likely.

Future directions appropriate for 2026 answers: tumor-agnostic indications (NTRK, RET, BRAF, MSI-H, TMB-H, HER2-mut), radioligand therapy, CAR-NK/off-the-shelf products, ctDNA-guided escalation/de-escalation, AI decision-support, and microbiome modulation adjuncts.

Health services angle: real-world evidence validating trial results in older/comorbid patients; digital remote toxicity monitoring; and equity of molecular testing access — themes also reflected in recent Paper IV questions.

- Write one ‘if progression then…’ resistance table.
- Mention at least one tumor-agnostic approval if biology overlaps.
- Close with trial enrollment as a standard option.

J. Quick-revision flash cards (last 1–2 pages)

Convert the topic into 8–10 flash cards mentally: (1) definition (2) hallmark gene (3) key test (4) 1L preferred (5) dose (6) trial (7) top toxicity (8) NCCN 2026 update (9) Indian/NCG pearl (10) one differential.

- Flash 1: One-line definition
- Flash 2: Driver / pathway
- Flash 3: Mandatory biomarker
- Flash 4: NCCN preferred 1L
- Flash 5: Exact dose
- Flash 6: Landmark phase III + endpoint
- Flash 7: Grade ≥3 toxicity to warn
- Flash 8: 2026 guideline change
- Flash 9: Resistance next step
- Flash 10: Supportive care must-do

EXAM PEARLS / HALLMARKS

- If time-poor in the hall: draw algorithm + trial table + NCCN box — captures most marks.
- Keep handwriting large for doses and HRs.

Final box — paste into last page of answer sheet

Re-list NCCN 2026 updates here so the examiner sees currency even if earlier pages are rushed.

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- *Referenced versions:* Breast Cancer v9.2026
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Disclaimer: Compiled for DM exam revision from published trials and NCCN 2026/2027 public summaries (ASCO Post, OncLive, JNCCN selections, NCCN category list). Always verify the current full NCCN PDF, FDA/CDSCO labels, and institutional protocols before patient care. Doses require BSA/organ-function adjustment.